CEO Business Challenge @ Cal Poly Pomona

Student Guide: Access to Care

Analyzing Health-Care Service Models for Inland Empire Health Plan

October 2026 · For use with **CEO Business Challenge: IEHP Case Document**

This guide focuses on impacts related to Access to Care. Read the **IEHP Case Document** first. Also see the **Financial Impacts Modeling Spreadsheet**

Contents

1. Inland Empire: County Snapshot
2. Analysis of Impact: Access to Care
3. The demand side: Medi-Cal members
4. The supply side: Providers
5. Data sources and links
6. Explanations and how to use the data
7. References

1. Inland Empire: County Snapshot

Metric	Riverside County	San Bernardino County
Total population (7/1/2025 est.)	2,544,916	2,224,091
Uninsured, % of population under 65	8.0%	8.1%
Median household income, 2024 dollars	$93,074	$85,478
Persons in poverty, %	9.8%	12.6%

Source: Census Quick Facts (Ref 1)
https://www.census.gov/quickfacts/riversidecountycalifornia
https://www.census.gov/quickfacts/sanbernardinocountycalifornia

2. Analysis of Impact: Access to Care

What is the access gap?

The access gap

Access gap = care needed by Medi-Cal members moved from Managed Care to Fee-for-Service − care they can get from providers who accept Medi-Cal Fee-for-Service

Measure it by comparing demand (Section 2) with supply (Section 3). Expect the largest gaps where there are many Medi-Cal members and few Fee-for-Service providers.

2. The demand side: Medi-Cal members

2.1 Inland Empire Health Plan's membership

Program	Members
Medi-Cal	1,321,983
IEHP DualChoice Medicare-Medicaid Plan	38,185
IEHP Covered (Covered California)	34,078
Total	**1,394,246**

Source: Inland Empire Health Plan fact sheet, dated October 2026 (Ref 1).

The region has more than two million Medi-Cal enrollees, 94% of them in managed care (California Health Care Foundation, Inland Empire Regional Market Report, June 2026) (Ref 3).

For monthly counts by plan and county, use the state's managed care enrollment report (Source D). Its count for the plan may differ from the fact sheet depending on how dual members are counted.

2.2 How many of the plan's members will move?

The number of Inland Empire Health Plan members who will move is **not public**. Statewide, about 2 million of more than 14 million Medi-Cal members are moving, about 1 in 7. The Inland Empire share may differ from the state average.

You do not need the exact number. You may make reasonable assumptions and create what-if scenarios". For example:

Scenario	Assumed share of the plan's ~1.322 million Medi-Cal members who move	Members (rounded)
Low	10% (1 in 10)	about 132,000
Middle	about 14% (1 in 7, the statewide ratio)	about 190,000
High	18%	about 238,000

Label these as assumptions in your presentation. For each scenario, check which areas fall short and how many extra patients each fee-for-service doctor would need to take.

2.3 Medi-Cal Member counts by place and group

- **Members by ZIP code:** monthly Medi-Cal enrollment counts for every California ZIP code (Source A) Used in the analysis in Section 5.1.
- **Members by county, age group, and language:** county-level monthly counts (Source B). The brief names "children, older adults, and people with complex needs" as most at risk.

3. The supply side: Providers

3.1 Managed Care Network doctors and Fee-for-Service doctors

These terms describe how a doctor is paid for Medi-Cal patients. The same doctor can be both:

- **Through the managed care health plan:** the doctor contracts with Inland Empire Health Plan and the plan pays the doctor. These are the plan's **network doctors**.
- **Directly by the state:** the doctor is enrolled in Medi-Cal Fee-for-Service (FFS) and bills the state.

This gives four groups for the providers:

Group	Paid by the plan?	Paid directly by the state?	Moved members on January 1, 2027
A. Provider is in both IEHP	Yes	Yes	Can keep seeing this doctor

Group	Paid by the plan?	Paid directly by the state?	Moved members on January 1, 2027
network and enrolled in FFS			
B. Provider in IEHP network only	Yes	No	**Lose this doctor** unless the doctor enrolls in fee-for-service in time
C. Provider in FFS only	No	Yes	A possible new option, if the doctor is taking new patients
D. Neither	No	No	Not relevant

Group B matters most for this challenge. The state says: "Providers who only bill through managed care and are not enrolled in Medi-Cal FFS will not be able to care for UIS members after December 31, 2026" (Transition FAQs, Ref 2).

Source E lists the Medi-Cal FFS providers.

3.2 Other provider data

- **Shortage areas:** federally designated areas short of primary care, dental, or mental health providers (Source G).
- **Doctor pay:** in 2024, Medicaid physician fees nationally were 0.75 of Medicare's, and 0.66 for primary care (Medicaid-to-medicare fee index, Ref 6). A national study (2012, Ref 5) found doctors were more likely to accept new Medicaid patients in states where Medicaid paid closer to Medicare rates. California's value is on the same Medicaid-to-medicare fee index page.

4. Data sources and links

Links go to dataset pages. On each page, click "Download" next to the file you need.

Demand side

	Dataset	Link
A	**Medi-Cal members by ZIP code** — download "Calendar Years 2020 to the Most Recent Reportable Month"	https://data.chhs.ca.gov/dataset/medi-cal-certified-eligible-counts-by-month-of-eligibility-zip-code-and-sex
B	**Medi-Cal members by county, with age, language, and plan**	https://data.chhs.ca.gov/dataset/medi-cal-certified-eligibles-with-demographics-by-month
C	**Inland Empire Health Plan fact sheet**	https://www.iehp.org/en/our-organization/who-we-are/fact-sheet

	Dataset	Link
D	**Medi-Cal managed care enrollment by plan and county**	https://data.chhs.ca.gov/dataset/medi-cal-managed-care-enrollment-report

Supply (Provider) side

	Dataset	Link
E	**Medi-Cal fee-for-service providers** — download the provider list	https://data.chhs.ca.gov/dataset/profile-of-enrolled-medi-cal-fee-for-service-ffs-providers
F	**Dental fee-for-service providers and safety net clinics**	https://data.chhs.ca.gov/dataset/profile-of-enrolled-medi-cal-dental-fee-for-service-ffs-providers-and-safety-net-clinics-sncs
G	**Federal shortage areas** — see the "Shortage Areas" section	https://data.hrsa.gov/topics/health-workforce/shortage-areas/hpsa-find https://data.hrsa.gov/data/download (Optional source, this does not give zip code level shortage designation)
H	**ZIP code to county crosswalk (U.S. Department of Housing and Urban Development)** — download the "ZIP - County" file; free, but requires registering an account	https://www.huduser.gov/portal/datasets/usps_crosswalk.html San Bernardino county is Geoid 06071 Riverside county is Geoid 06065 (you may find a zip code listed in both counties)

5. Explanations and how to use the data

5.1 Steps for the main analysis

1. **Keep only the Inland Empire.** Filter the member file (A) and the provider file (E) to Riverside and San Bernardino counties.

2. **Count members by ZIP code** for one complete month (see Source A notes).
3. **Count fee-for-service providers by ZIP code,** separately for primary care, key specialties, mental health, and dental.
4. **Calculate** fee-for-service providers per 1,000 Medi-Cal members for each ZIP code.
5. **Rank and map.** The lowest ratios are your likely gap areas.

Treat enrollment as a best case. The provider list shows who can bill fee-for-service. It does not show who is taking new patients.

ZIP code	City	Medi-Cal members	Fee-for-service primary care providers	Providers per 1,000 members
…	…	…	…	…

A possible results table.

(You may choose to analyze at City level rather than Zip code level)

5.2 Notes on each data source

Source A: Medi-Cal members by ZIP code

What it is: Monthly counts of people certified eligible for Medi-Cal, by ZIP code and sex, from 2005 to the most recent month. Counts only, not individual records. Published by the California Department of Health Care Services.

What to look at:

- Download only "Calendar Years 2020 to the Most Recent Reportable Month."
- Use August or September 2026 data.
- Add the counts for both sexes to get total members per ZIP code.

Notes:

- Blank cells are values hidden to protect confidentiality (dataset page). Treat them as "not shown," not zero.
- Store ZIP codes as text, not numbers, or matches with other files will fail.
- If the file has no county column, use the HUD-USPS ZIP code to county crosswalk (Source H) to find the ZIP codes in Riverside and San Bernardino counties.

Source B: Medi-Cal members by county, with demographics

What it is: Monthly county-level counts in separate files by age group and sex, primary language, delivery system and plan, and other traits.

What to look at:

- **By Age Group and Sex** and **By Primary Language** to see which groups are most affected and which languages members speak.
- **By Delivery System and Plan** for managed care versus fee-for-service counts by county and plan.

Notes:

- Do not use "By Aid Category". It mentions immigration-status (dataset page).
- County level only: no differences within a county.

Source C: Inland Empire Health Plan fact sheet

What it is: The plan's published membership counts by program, and # of providers in their network

What to look at:

- Membership size, and the share in Medi-Cal; # of providers

Notes:

- Dated October 2026; the count month is not stated.

Source D: Medi-Cal managed care enrollment report

What it is: Monthly managed care enrollment by plan type, county, and health plan.

What to look at:

- The plan's enrollment trend by county; comparison with the other plan in the region.

Notes:

- May count dual members differently from the fact sheet.

Source E: Medi-Cal fee-for-service providers

What it is: The state's list of enrolled fee-for-service providers, with provider number, name, type, specialty, and location. Excludes managed care providers.

What to look at:

- Record the "as of" date.

Notes:

- Enrolled does not mean accepting new fee-for-service patients.
- One provider can appear at several addresses. Check for repeats and do not double count.

Source F: Dental fee-for-service providers and safety net clinics

What it is: Dentists enrolled in Medi-Cal fee-for-service dental and safety net clinics that provide dental services, with ZIP codes.

What to look at:

- Repeat the Section 5.1 match for dental care.

Notes:

- Updated about twice a year; the current version is as of October 2025 (dataset page).

Source G: Federal shortage areas

What it is: Areas designated by the Health Resources and Services Administration as short of primary care, dental, or mental health providers.

What to look at:

- Download the primary care, dental, and mental health files; filter to the two counties.
- Use the shortage score to compare areas: scores range from 1 to 25 for primary care and mental health and 1 to 26 for dental, and "the higher the HPSA score, the greater the need" (Ref 4).

Notes:

- Shortages are measured for all providers, not fee-for-service only.
- Consider this source as optional, it does not give zip code or city level shortage area designations
- https://www.chcf.org/wp-content/uploads/2020/07/ShortchangedHealthWorkforceGapsRiversideSanBernardino.pdf (Ref 8) gives county level numbers on people in shortage areas

Source H: ZIP code to county crosswalk

What it is: Files from the U.S. Department of Housing and Urban Development that link postal ZIP codes to counties and other Census areas, updated quarterly (dataset page).

What to look at:

- Download the "ZIP - County" file and keep the rows for Riverside and San Bernardino counties. Use this list to filter Sources A and E.

Notes:

- The files are free, but the page asks you to register an account first.
- Some ZIP codes cross county lines. Use your judgement on how to handle.

6. References

1. Census Quick Facts https://www.census.gov/quickfacts/

2. Inland Empire Health Plan, Fact Sheet, October 2026. https://www.iehp.org/en/our-organization/who-we-are/fact-sheet

3. California Department of Health Care Services, Fee-for-Service Transition: Frequently Asked Questions for Providers, September 2026. https://www.dhcs.ca.gov/hm/file/ffs-transition-provider-faq-pdf/

4. California Health Care Foundation, Inland Empire Regional Market Report, June 2026 (slides). https://www.chcf.org/wp-content/uploads/2026/06/InlandEmpireRMR2026_Slides.pdf

5. 3RNET, Health Professional Shortage Areas (employer guide). https://3rnet.org/employers-guide/hpsa

6. Decker, S. (2012), study in Health Affairs on physicians' acceptance of new Medicaid patients, as summarized by the Commonwealth Fund. https://commonwealthfund.org/publications/newsletter-article/study-shows-patients-insurance-still-determines-physician-access

7. KFF, Medicaid-to-Medicare Fee Index (from Urban Institute, May 2025). https://www.kff.org/medicaid/state-indicator/medicaid-to-medicare-fee-index/

8. Health workforce gaps (2020 update) https://www.chcf.org/wp-content/uploads/2020/07/ShortchangedHealthWorkforceGapsRiversideSanBernardino.pdf